NCCP 00623 · clinician draft

Bevacizumab + capecitabine

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab + capecitabine · active treatment cycle

Active medicines: Capecitabine + Bevacizumab

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Metoclopramide	10 mg PO up to three times daily as required	Only when needed, as prescribed One option for oral minimal-low risk; check renal function, contraindications, interactions and duration. Do not add all alternatives concurrently.

Additional phase-specific support

- The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.
- Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.
- Where prescribed under the capecitabine source: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day; follow the written local escalation plan. Do not copy the irinotecan high-dose schedule.
- No routine bevacizumab hypersensitivity premedication unless previous reaction or a specific local plan. Review blood pressure, proteinuria, bleeding and wound healing separately from nausea prevention.
- If taking warfarin: INR weekly until the warfarin dose is stable, then before each cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00623 v3a · NCCP Classification V6 (2025), oral minimal-to-low risk Table 5

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required